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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Σ, 87

Name : HODA FATHY IBRAHEM

Date : 15-02-2026 10:47

Branch : Portable

Medical No. : GRI-12341

Ref.Physician Prof : CT CAP+C

Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN& PELVIS WITH CORONAL RECONSTRUCTION: -

As compared to previous study dated 17/9/2025, the current study revealed

I CHEST

- Left mastectomy with clear operative bed.
- Clear both lung fields. No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.

II ABDOMEN& PELVIS

- The liver is of average size with homogenous CT texture. Newly developed hypodense focal lesion seen subcapsular at segment VI measuring 19x13mm
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

III EXAMINED SKELETON

- Progressive course of the osseous lesions with multiple predominately lytic lesions involving the right iliac bone, right ischial tuberosity, acetabulum, left ala of the sacrum
- Multiple vertebral lytic& sclerotic lesions

Opinion

Newly developed liver focal lesion

Progressive course of the osseous lesions

(Progressive course as regarding the previous study)

DRAHERMEEN RAMADAN.MD



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